SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

TCHP with Phesgo

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

HER2-positive neoadjuvant breast cancer · NCCP 00789 · version 3 · 21-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
Shrink the cancer before surgery and contribute to reducing recurrence risk as part of a complete early-breast plan.

Schedule
Six 21-day cycles before surgery: Phesgo, docetaxel and carboplatin on Day 1. Surgery and pathology determine the postoperative HER2 medicine and remaining course.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Duration and treatment setting discussed
- ☐ Supportive medicines and urgent contact advice discussed
- ☐ Common, serious and lasting agent-specific risks discussed below
- ☐ Correct HER2 medicine and formulation confirmed
- ☐ Heart and other required monitoring explained
- ☐ Pregnancy / contraception / breastfeeding discussed where relevant
- ☐ Companion cancer medicines reviewed using their own information

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Docetaxel
TAXANE CHEMOTHERAPY

- ☐ Low blood counts, infection and sepsis
- ☐ Hair loss, mouth soreness and fatigue
- ☐ Numbness / tingling; sometimes lasting
- ☐ Fluid retention and nail changes
- ☐ Allergic infusion reaction

Carboplatin
PLATINUM CHEMOTHERAPY

- ☐ Low blood counts, infection and bleeding
- ☐ Nausea, vomiting and fatigue
- ☐ Allergic reaction, sometimes after previous exposure
- ☐ Kidney or electrolyte changes

Pertuzumab
HER2-TARGETED ANTIBODY

- ☐ Diarrhoea and dehydration
- ☐ Injection / allergic reaction
- ☐ Heart pumping-function reduction
- ☐ Rash and fatigue

Trastuzumab
HER2-TARGETED ANTIBODY

- ☐ Heart pumping-function reduction / heart failure
- ☐ Injection or allergic reaction
- ☐ Diarrhoea and fatigue
- ☐ Rare serious lung effects

Discuss the aim, alternatives, uncertain benefit, common and serious risks, heart function, diarrhoea, injection reaction, fertility and pregnancy precautions. Effective contraception is required during Phesgo and for 7 months after the last dose; review the full combination and breastfeeding restrictions with the team. After surgery, your team reviews the pathology and chooses the next HER2 medicine. The usual plan aims to complete about one year of HER2 treatment from the first dose. Remaining invasive cancer can change the medicine offered. The selected treatment has its own blood tests and supportive medicines.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00789 v3.

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